

Intimate Care policy

Extra Time practitioners, who work with our young children or children/young people with/without specific needs, do realise that the issue of intimate care is a difficult one and will require practitioners to be respectful of children's needs.

Intimate care can be defined as care tasks of an intimate nature, associated with bodily functions, body products and personal hygiene which demand direct or indirect contact with or exposure of the genitals. Examples include care associated with continence and menstrual management as well as more ordinary tasks such as help with washing or bathing.

Children's dignity will be preserved and a high level of privacy, choice and control will be provided to them. Practitioners who provide intimate care to children have a high awareness of child protection issues. Staff behaviour is open to scrutiny and staff at extra time work in partnership with parents/carers to provide continuity of care to children/young people wherever possible.

Extra time staff discuss full personal safety as part of Personal, Social and Health Education as and where necessary, to all children as appropriate to their developmental level and degree of understanding. This work is shared with parents who are encouraged to reinforce the personal safety messages within the home.

Extra time is committed to ensuring that all staff responsible for the intimate care of children, will undertake their duties in a professional manner at all times. The management recognises that there is a need to treat all children with respect when intimate care is given. No child should be attended to in a way that causes distress or pain.

All children who require intimate care are treated respectfully at all times; the child's welfare and dignity are of paramount importance.

Practitioners who provide intimate care will be trained to do so (including Child Protection/ Safeguarding and Health and Safety training in moving and handling) and are fully aware of best practice. Apparatus will be provided to assist with children who need special arrangements following assessment from physiotherapist/ occupational therapist and families as required.

Practitioners will be supported to adapt their practice in relation to the needs of individual children considering developmental changes such as the onset of puberty and menstruation. (Please see maturing child procedure below). Wherever possible, practitioners who are involved in the intimate care of children/young people will ensure that they safeguard both staff and children/young people involved.

There is careful communication with each child who needs help with intimate care in line with their preferred means of communication (verbal, symbolic, etc.) to discuss the child's needs and preferences. The child is aware of each procedure that is carried out and the reasons for it.

Practitioners will encourage each child to do as much for themselves as they can. This may mean, for example, giving the child responsibility for washing themselves. Individual intimate care plans will be drawn up for children as appropriate to suit the circumstances of the child. These plans will include a full risk assessment to address issues such as moving and handling, personal safety of the child and the carer and health.

Each child's right to privacy will be respected. Careful consideration will be given to each child's situation to determine how many practitioners may need to be present when a child needs help with intimate care. Where possible, one child will be cared for by two adults, however, if there are only two staff members on duty at any one time, there will be a need for only one member of staff to be present. If this is the case, the reasons why there is only one team member supporting the child should be clearly documented. We know that we put ourselves in vulnerable positions in cases like this, this is why we have written care plans to cover ourselves.

Wherever possible the child will not be cared for by the same adult on a regular basis;

There will be a Rota of practitioners known to the child who will take turns in providing care. This will ensure, as far as possible, that over-familiar relationships are discouraged from developing, while at the same time guarding against the care being carried out by a succession of completely different carers.

Parents/carers will be involved with their child's intimate care arrangements on a regular basis; a clear account of the agreed arrangements will be recorded on the child's care plan. The needs and wishes of children and parents will be carefully considered alongside any possible constraints; e.g. staffing and equal opportunities legislation.

Each child/young person will have an assigned senior member of staff to act as an advocate to whom they will be able to communicate any issues or concerns that they may have about the quality of care they receive.

Child Protection Procedures and Inter-Agency Child Protection procedures will be accessible to practitioners and adhered to.

Where appropriate, all children will be taught personal safety skills carefully matched to their level of development and understanding.

If a practitioner has any concerns about physical changes in a child's presentation, e.g. marks, bruises, soreness etc. s/he will immediately report concerns to the appropriate manager/designated person for child protection. A clear record of the concern will be completed and referred to social care and/or the police if necessary. Parents will be asked for their consent or informed that a referral is necessary prior to it being made unless doing so is likely to place the child at greater risk of harm.

If a child becomes distressed or unhappy about being cared for by a particular member of staff, the matter will be looked into and outcomes recorded. Parents/carers will be contacted at the earliest opportunity as part of this process in order to reach a resolution. Staffing schedules will be altered until the issue(s) are resolved so that the child's needs remain paramount. Further advice will be taken from outside agencies if necessary.

Maturing child procedure

When a parent or legal guardian come into register their female child, and she is of an age where menstruation can start (9-16yrs), the practitioner booking the child in, will discuss with the parent / legal guardian what to do if their child starts menstruating whilst in our care, whether they want us to tell her what is happening, or whether they can discuss the issue with her before she starts to attend.

If the parent / legal guardian gives the practitioner permission to discuss with the child, what is happening to them, the explanation will be given to the child by a practitioner whom the child feels comfortable with, and ensuring they understand how to use sanitary wear and how to dispose of them whilst in the building.

Sanitary wear will be in stock for emergency cases.

If a child makes an allegation against a practitioner, all necessary procedures will be followed

This policy was reviewed on: 09/03/2026 written by: A Grosse (owner)

I the undersigned, am signing to say that I have reviewed this policy with the team and they will adhere to the policy at all times.

L.Nordon-guest: Date: